SEA HEARTS

World Health Organization
South-East Asia Region

SEAHEARTS 2030:
Monitoring Framework

October 2025

SEAHEARTS 2030:
Monitoring Framework

October 2025

World Health Organization
South-East Asia Region

SEAHEARTS 2030: Monitoring Framework
ISBN: 978-92-9022-186-9

© World Health Organization 2025

Some rights reserved. This work is available under the Creative Commons Attribution-NonCommercial-ShareAlike 3.0 IGO licence (CC BY-NC-SA 3.0 IGO; https://creativecommons.org/licenses/by-nc-sa/3.0/igo).

Under the terms of this licence, you may copy, redistribute and adapt the work for non-commercial purposes, provided the work is appropriately cited, as indicated below. In any use of this work, there should be no suggestion that WHO endorses any specific organization, products or services. The use of the WHO logo is not permitted. If you adapt the work, then you must license your work under the same or equivalent Creative Commons licence. If you create a translation of this work, you should add the following disclaimer along with the suggested citation: "This translation was not created by the World Health Organization (WHO). WHO is not responsible for the content or accuracy of this translation. The original English edition shall be the binding and authentic edition".

Any mediation relating to disputes arising under the licence shall be conducted in accordance with the mediation rules of the World Intellectual Property Organization.

Suggested citation: SEAHEARTS 2030: Monitoring Framework. New Delhi: World Health Organization, Regional Office for South-East Asia; 2025. Licence: CC BY-NC-SA 3.0 IGO.

Cataloguing-in-Publication (CIP) data. CIP data are available at http://apps.who.int/iris.

Sales, rights and licensing. To purchase WHO publications, see http://apps.who.int/bookorders. To submit requests for commercial use and queries on rights and licensing, see http://www.who.int/about/licensing.

Third-party materials. If you wish to reuse material from this work that is attributed to a third party, such as tables, figures or images, it is your responsibility to determine whether permission is needed for that reuse and to obtain permission from the copyright holder. The risk of claims resulting from infringement of any third-party-owned component in the work rests solely with the user.

General disclaimers. The designations employed and the presentation of the material in this publication do not imply the expression of any opinion whatsoever on the part of WHO concerning the legal status of any country, territory, city or area or of its authorities, or concerning the delimitation of its frontiers or boundaries. Dotted and dashed lines on maps represent approximate border lines for which there may not yet be full agreement. The mention of specific companies or of certain manufacturers' products does not imply that they are endorsed or recommended by WHO in preference to others of a similar nature that are not mentioned. Errors and omissions excepted, the names of proprietary products are distinguished by initial capital letters.

All reasonable precautions have been taken by WHO to verify the information contained in this publication. However, the published material is being distributed without warranty of any kind, either expressed or implied. The responsibility for the interpretation and use of the material lies with the reader. In no event shall WHO be liable for damages arising from its use.

World Health Organization
South-East Asia Region

DECISION

OF THE

WHO REGIONAL COMMITTEE FOR SOUTH-EAST ASIA

SEA/RC78(7)

Extending and expanding the Regional Milestones of SEAHEARTS: an initiative to reduce CVD burden in the WHO South-East Asia Region

The Regional Committee,

HAVING CONSIDERED the updates on specific Regional Committee resolutions and documents presented to its Seventy-eighth Session, including the Progress Report on its resolution SEA/RC76/R5 titled '*SEAHEARTS: Accelerating prevention and control of cardiovascular diseases in the WHO SE Asia Region*'.

NOTING the progress made over the implementation of SEAHEARTS interventions including on the four Regional Milestones1 for 2025, and the suggested way forward therein,

RECOGNIZING that the burden of cardiovascular diseases in the South-East Asia Region is expected to continue to increase due to population growth, ageing and high prevalence of risk factors,

RECOGNIZING FURTHER that the Region still faces considerable challenges in addressing the major CVD risk factors, particularly uncontrolled hypertension, along with diabetes, tobacco use, unhealthy diet (including high salt intake and consumption of trans-fatty acids), and physical inactivity; that the interventions aimed at achieving the expanded Milestones require comprehensive and integrated approaches; and that they have strong correlations and complementarities with the interventions under the Global NCD Targets as well as the SDG Targets set for 2030,

1 SEAHEARTS: an initiative to reduce CVD burden in the WHO South-East Asia Region. New Delhi: WHO Regional Office for South-East Asia. https://cdn.who.int/media/docs/default-source/searo/ncd/seahearts-reduce-cardiovasculardisease-burden-in-the-sear.pdf - accessed 2 September 2025.

DECIDES:

1. **TO EXTEND** the timeline of the four Regional Milestones of the SEAHEARTS Initiative for another 5 years, till 2030; and

2. **TO REQUEST** the Regional Director to:

   (a) convene technical consultation(s) with Member States and key stakeholders to develop the proposed extended and expanded regional SEAHEARTS Milestones for 2030, encompassing tobacco control; salt reduction; elimination of trans-fatty acids; and most urgently, improved detection, treatment and control of hypertension and diabetes as primary risk factors for cardiovascular diseases;

   (b) support Member States in developing national and subnational roadmaps with targets for the implementation of WHO technical packages, HEARTS[2] (technical package for CVD management in primary health care), MPOWER[3] (measures to reduce tobacco demand under the WHO Framework Convention on Tobacco Control (FCTC), SHAKE[4] (technical package for salt reduction) and REPLACE[5] (technical package for eliminating industrially produced trans-fatty acids), in order to achieve the SEAHEARTS 2030 milestones with particular emphasis on reducing uncontrolled hypertension through population-level and clinical interventions; and

   (c) report the progress, achievements and challenges on the implementation of this decision to the Regional Committee sessions in 2026, 2028 and 2030.

---

2 HEARTS: a technical package for cardiovascular disease management in primary health care. Geneva: World Health Organization, 2020. https://www.who.int/publications/i/item/9789240001367

3 MOPWER: a technical package for tobacco control. Geneva: World Health Organization, 2023. https://www.who.int/initiatives/mpower

4 SHAKE: a technical package for salt reduction. Geneva: World Health Organization, 2016. https://apps.who.int/iris/bitstream/handle/10665/250135/9789241511346-eng.pdf;sequence=1

5 REPLACE: an action package to eliminate industrially produced trans-fatty acids. Geneva: World Health Organization, 2021. https://www.who.int/publications/i/item/9789240021105

Full document: *https://www.who.int/southeastasia/about/governance/regional-committee/seventy-eighth-session/resolutions-and-decisions-of-rc78*

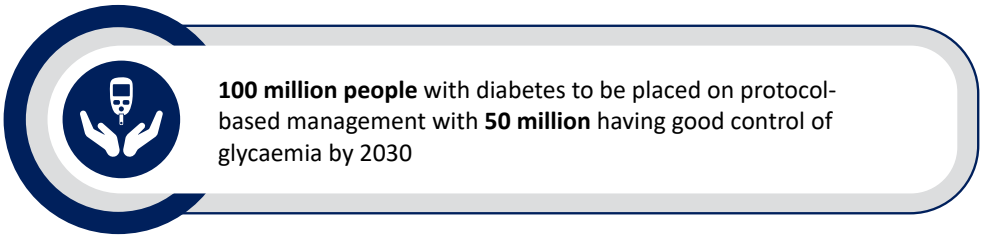
Contents

What are SEAHEARTS 2030 milestones? 1

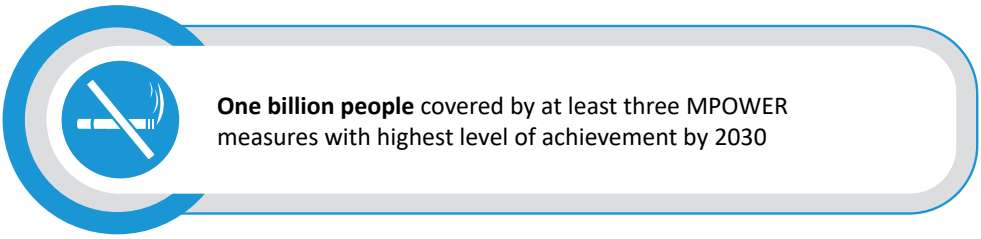
Scope and purpose of the SEAHEARTS 2030 Monitoring Framework 1

Indicator definitions and data requirements of SEAHEARTS 2030 milestones 3

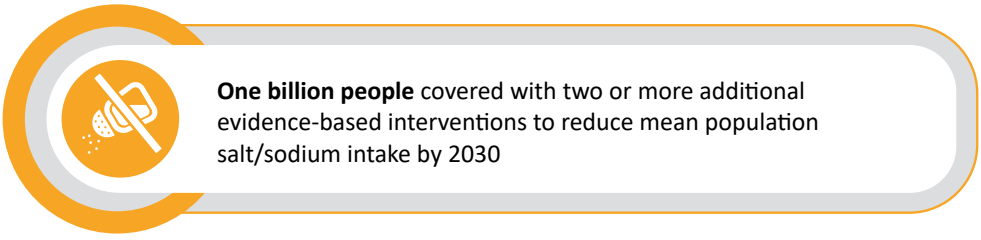
5



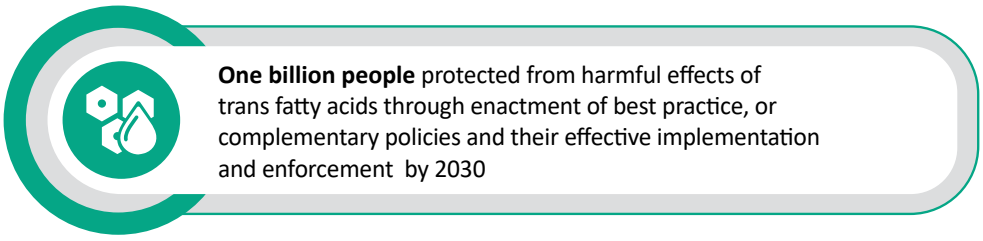
7



9



13



15

Monitoring the progress of the implementation of SEAHEARTS interventions at primary health care facilities 17

What are SEAHEARTS 2030 milestones?

At the 78th Regional Committee of the WHO South-East Asia region, the Member States decided to the timeline of the four Regional Milestones of the SEAHEARTS Initiative for another 5 years, till 2030 and to report the progress, to the Regional Committee sessions in 2026, 2028 and 2030

The set of extended and expanded regional SEAHEARTS Milestones for 2030, are,

A. 100 million people with hypertension to be placed on protocol-based management with 60 million having controlled blood pressure by 2030

B. 100 million people with diabetes to be placed on protocol-based management with 50 million having good control of glycaemia by 2030

C. One billion people covered by at least three MPOWER measures with highest level of achievement by 2030

D. One billion people covered with two or more additional evidence-based interventions to reduce mean population salt/sodium intake by 2030

E. One billion people are protected from harmful effects of trans fatty acids through enactment of best practice, less restrictive or complementary policies and effective implementation and enforcement of policies by 2030

Scope and purpose of the SEAHEARTS 2030 Monitoring Framework

The SEAHEARTS 2030 Monitoring Framework comprises indicator definitions and measurement criteria for each of the five SEAHEARTS Milestones 2030. The indicator definitions for milestones on hypertension and diabetes are aligned with the WHO NCD Facility-Based Monitoring Framework[1]. The measurement criteria for the milestones on risk factors are aligned to key global benchmarks, including the Global Tobacco

1 Noncommunicable disease facility-based monitoring guidance: framework, indicators and application. Geneva: World Health Organization; 2022. https://iris.who.int/server/api/core/bitstreams/610c0cda-923d-4888-882f-926311d4ab22/content

Epidemic Report[2], the Sodium Score Card[3] and the Trans-Fatty Acid Score Card of the WHO Global Database on the Implementation of Food and Nutrition Action[4]. In addition to the milestones, a set of process indicators to monitor the progress of the implementation of technical packages WHO HEARTS and HEARTS-D are also included. It is proposed to monitor the progress biannually, through data reported by the focal points of Ministries of Health.

The digital SEAHEARTS Monitoring Platform[5] will be configured to track and report the SEAHEARTS milestones 2030, national targets countries have committed to, case studies, best practices, key interventions and learning resources. It will serve as a hub for knowledge-sharing, and documentation of learning and regional collaboration.

2 World Health Organization (2025). WHO report on the global tobacco epidemic, 2025: warning about the dangers of tobacco. World Health Organization. https://iris.who.int/handle/10665/381685

3 World Health Organization (2023). WHO global report on sodium intake reduction. World Health Organization. https://iris.who.int/handle/10665/366393

4 GIFNA Home. https://gifna.who.int/

5 Home page | ACCELERATING PREVENTION AND CONTROL OF CARDIOVASCULAR DISEASES IN THE SOUTH-EAST ASIA REGION

Indicator definitions and data requirements of

SEAHEARTS 2030 milestones

A. 100 million people with hypertension to be placed on protocol-based management with 60 million having controlled blood pressure by 2030

Indicator definition

A1. Number of people with hypertension placed on protocol-based management

The count of patients who have been registered for hypertension treatment at facility/clinic at any point of time in the past, excluding

- those who have not visited the clinic for a period of one year prior to the point of assessment defined as lost to follow up

Data requirements

- The number should NOT include multiple counts of a single person and requires longitudinal records of clinic visits and the BP at each visit for each patient.
- Electronic systems using unique IDs can be configured to generate the data on this indicator.

Indicator definition

A2. Number of people registered for hypertension treatment in the facility with controlled blood pressure

The count of patients registered for hypertension treatment in the facility whose blood pressure was controlled at the last clinic visit, excluding

- those who were registered in the clinic within a period of three months prior to the point of assessment defined as new registrants
- those who have not visited the clinic for a period of one year prior to the point of assessment defined as lost to follow up

Data requirements

- The number should NOT include multiple counts of a single person and requires longitudinal records of clinic visits and the BP at each visit for each patient.
- Electronic systems using unique IDs can be configured to generate the data on this indicator

- Definition of the controlled blood pressure should be based on the
  - National treatment guidelines

    OR
  - WHO guidelines which considers BP is controlled when:
    - Systolic BP(SBP) <140 mmHg and diastolic BP(DBP) <90 mmHg
    - SBP <130 mmHg among people with history of CVD
    - SBP <130 mmHg among high-risk people with hypertension, i.e., those with high CVD risk, diabetes mellitus, chronic kidney disease (CKD)

	Primary	Secondary	Tertiary
Total number of health facilities in the public sector			
The number of facilities which contributed to the count of persons on protocol-based management for hypertension			
The number of facilities which contributed to the count of number with controlled blood pressure			

B. 100 million people with diabetes to be placed on protocol-based management with 50 million having good control of glycaemia by 2030

Indicator definition

B1. Number of people with diabetes placed on protocol-based management

The count of patients who have been registered for diabetes treatment at facility/clinic at any point of time in the past, excluding

- those who have not visited the clinic for a period of one year prior to the point of assessment defined as lost to follow-up

Data requirements

- The number should NOT include multiple counts of a single person and requires longitudinal records of clinic visits and the records of glycaemic control for each patient.
- Electronic systems using unique IDs can be configured to generate the data on this indicator.

Indicator definition

B2. Number of people registered for diabetes treatment in the facility having good control of glycaemia

The count of patients registered for diabetes treatment in the facility having good control of glycaemia at the last clinic visit, excluding

- those who were registered in the clinic within a period of three months prior to the point of assessment defined as new registrants
- those who have not visited the clinic for a period of one year prior to the point of assessment defined as lost to follow up

Data requirements

- The number should NOT include multiple counts of a single person and requires longitudinal records of clinic visits and the records of glycaemic control for each patient.
- Electronic systems using unique IDs can be configured to generate the data on this indicator.
- Definition of the controlled blood pressure should be based on
  - HbA1c <8% (64 mmol/mol)

    OR
  - fasting plasma glucose (FPG) <7.0 mmol/l or <126 mg/dl

	Primary	Secondary	Tertiary
Total number of health facilities in the public sector			
The number of facilities which contributed to the count of persons on protocol-based management for diabetes			
The number of facilities which contributed to the count of number having good control of glycaemia			

C. One billion people covered by at least three MPOWER measures with highest level of achievement by 2030

Indicator definition

The count of the people will be the total population* of the countries in the Region which fulfill the criteria for highest level of achievement for at least three WHO MPOWER measures for tobacco control.

* World population prospects: 2024 revision. New York (NY): United Nations, United Nations, 2024 (https://population.un.org/wpp/)

Country	Population	Country	Population
Bangladesh	173,562,000	Myanmar	54,500,000
Bhutan	792,000	Nepal	29,651,000
DPR Korea	26,499,000	Sri Lanka	23,104,000
India	1,450,936,000	Thailand	71,668,000
Maldives	528,000	Timor-Leste	1,401,000

Measurement criteria for SEAHEARTS indicator

WHO MPOWER measures		Indicator summary	Criteria to be fulfilled by the countries for highest level of achievement**
M	Monitor tobacco use and prevention policies	Monitoring tobacco use prevalence data and various tobacco use prevention policies, in context of both adults and youth	Having recent[i], representative[ii] and periodic[iii] data for prevalence of tobacco use for both adults and youth
P	Protect people from tobacco smoke	Smokefree legislations leading to smokefree environments	All public places completely smoke-free[iv] (or at least 90% of the population covered by complete subnational smoke-free legislation)

WHO MPOWER measures		Indicator summary	Criteria to be fulfilled by the countries for highest level of achievement**
O	Offer help to quit tobacco use	Treatment of tobacco dependence through provisioning tobacco cessation services	National quit line, and both NRT and some cessation services cost- covered[v]
W	Warn about the dangers of tobacco	Warning the population about deleterious health effects of tobacco through implementation of health warnings on packages of tobacco products	Large warnings with all appropriate[vi] characteristics on cigarette packages[vii]
		Warning the population about deleterious effects of tobacco through implementation of mass media anti-tobacco campaigns	National anti-tobacco campaign conducted with at least seven appropriate characteristics[viii] including airing on television and/or radio
E	Enforce bans on tobacco advertising, promotion and sponsorship (TAPS)	Ban on ALL forms of TAPS	Ban on all forms of direct[ix] and indirect[x] advertising (or at least 90% of the population covered by subnational legislation completely banning tobacco advertising, promotion and sponsorship)
R	Raise taxes on tobacco	Tobacco taxation: share of total taxes in the retail price of the most widely sold brand of cigarettes	Share of total taxes be ≥75% of the retail price of the most widely sold brand of cigarettes

** The detail of each indicator is provided in the technical note provided in the WHO Global Tobacco Epidemic Report 2023 - WHO report on the global tobacco epidemic, 2023: protect people from tobacco smoke

Criteria for highest level of achievement (MPOWER)

Monitoring of tobacco use and prevention policies

i. Recent data (from the past five years or newer).

ii. Survey sample representative of the national population.

iii. Collected at least every 5 years.

Smokefree environments

iv. Complete indoor smoke free environment at all times, in all the facilities of each of the following eight places:
   - health care facilities;
   - educational facilities other than universities;
   - universities;
   - governmental facilities;
   - indoor offices and workplaces not considered in any other category;
   - restaurants or facilities that serve mostly food;
   - cafés, pubs and bars or facilities that serve mostly beverages;
   - public transport

Cessation services

v. Tobacco cessation support available in and of the following places: health clinics or other primary care facilities, hospitals, office of health professional, the community or other settings

Warning labels

vi. Appropriate characteristic of tobacco package warnings:
   - specific health warnings mandated;
   - appearing on individual packages as well as on any outside packaging and labelling used in retail sale;
   - describing specific harmful effects of tobacco use on health;
   - are large, clear, visible and legible (e.g. specific colours and font style and sizes are mandated);
   - rotate;
   - include pictures or pictograms;
   - written in (all) the principal language(s) of the country.

vii. Average of front and back of the package is at least 50%.

Anti-tobacco mass media campaigns

viii. Appropriate characteristics, which signify the use of a comprehensive communication approach:

- The campaign was part of a comprehensive tobacco control programme.
- Before the campaign, research was undertaken or reviewed to gain a thorough understanding of the target audience.
- Campaign communication materials were pre-tested with the target audience and refined in line with campaign objectives.
- Airtime (radio, television) and/ or placement (billboards, print advertising, etc.) were obtained by purchasing or securing it using either the organization's own internal resources or an external media planner or agency (this information indicates whether the campaign adopted a thorough media planning and buying process to effectively and efficiently reach its target audience).
- The implementing agency worked with journalists to gain publicity or news coverage for the campaign.
- Process evaluation was undertaken to assess how effectively the campaign had been implemented.
- An outcome evaluation process was implemented to assess campaign impact.
- The campaign was aired on television and/or radio. Bans on tobacco advertising, promotion and sponsorship (TAPS)

ix. Direct advertisement ban:

- national television and radio;
- local magazines and newspapers;
- billboards and outdoor advertising;
- point of sale (indoor)

x. Indirect advertising bans:

- free distribution of tobacco products in the mail or through other means;
- promotional discounts;
- non-tobacco goods or services identified with tobacco brand names (brand stretching);
- brand names of non-tobacco products used for tobacco products (brand sharing);
- appearance of tobacco brands (product placement) or tobacco products in television and/or films; sponsorship (contributions and/or publicity of contributions).

D. One billion people covered with two or more additional evidence-based interventions to reduce mean population salt/sodium intake by 2030

Indicator definition

The count of the people will be the total population* of the countries in the Region which fulfill the criteria for two or more additional evidence-based interventions for reducing mean population intake of salt/sodium.

* World population prospects: 2024 revision. New York (NY): United Nations, United Nations, 2024 (https://population.un.org/wpp/)

Country	Population	Country	Population
Bangladesh	173,562,000	Myanmar	54,500,000
Bhutan	792,000	Nepal	29,651,000
DPR Korea	26,499,000	Sri Lanka	23,104,000
India	1,450,936,000	Thailand	71,668,000
Maldives	528,000	Timor-Leste	1,401,000

Measurement criteria for SEAHEARTS indicator

WHO's evidence-based measures for reducing mean population intake of salt/sodium[6]	Criteria to be fulfilled by the countries	Source/ validation[7]
Food reformulation policies that set scope and criteria for maximum limits or targets for the sodium content in prepackaged food	Adopted and set national/ subnational targets for maximum sodium content levels in one or more selected food categories,[8] with timelines for meeting the targets (voluntary or legislative approach)	Country Capacity Survey report OR Country submission of evidence: • legislation, • regulation, • Government gazette notification • Government directive
Front of Pack Labelling (FOPL) as part of comprehensive food and nutrition labelling	Adopted policies that set scope and criteria to • list and declare sodium on packaged food, • regulate the use of sodium-related claims on packaged food and • use front-of-pack labelling (FOPL) to provide interpretive information about sodium content.	
Food procurement and service policies	Adopted policies that set scope and criteria for sodium content in food served or sold in public settings and/ or procured by government for food programmes.	
Food marketing restrictions	Adopted policies that set scope and criteria for restricting food marketing to protect children from its harmful impact on diet.	
Behaviour change communication to reduce salt intake in the context of overall healthy diets	A set of communication activities designed to change behaviours to reduce consumption of salt and high sodium food have been implemented at national or subnational level in the past two years	
Taxation of unhealthy foods	Adopted policies that set scope and criteria for taxes on unhealthy food, including high-sodium food.	

6 While Surveillance is a part of SHAKE, it is for monitoring, and not included as a best buy intervention for sodium reduction (includes measuring population salt consumption patterns; sodium content of food; monitor and evaluate the impact of the salt reduction programme).

7 WHO global report on sodium intake reduction obtains data from NCD CCS, therefore NCD CCS is the source of validation

8 Regional sodium benchmarks available

E. One billion people protected from harmful effects of trans fatty acids through enactment of best practice or complementary policies and effective implementation and enforcement of policies by 2030

The count of the people will be the total population* of the countries in the Region which fulfill the criteria for being protected from the harmful effects of trans-fatty acids through enactment of best practice, less restrictive or complementary policies and effective implementation and enforcement of policies of WHO REPLACE.

* World population prospects: 2024 revision. New York (NY): United Nations, United Nations, 2024 (https://population.un.org/wpp/)

Country	Population	Country	Population
Bangladesh	173,562,000	Myanmar	54,500,000
Bhutan	792,000	Nepal	29,651,000
DPR Korea	26,499,000	Sri Lanka	23,104,000
India	1,450,936,000	Thailand	71,668,000
Maldives	528,000	Timor-Leste	1,401,000

Measurement criteria for SEAHEARTS indicator

Trans fatty acid elimination measures for SEAHEARTS	Criteria to be fulfilled by the countries	Source/validation[9]
Best-practice TFA policy passed	Having mandatory national limit of 2 g of industrially produced TFA per 100 g of total fat in all foods OR Mandatory national ban on the production or use of PHO as an ingredient in all foods	Country legislation or regulation, or Gazette notification
Other Complementary measures effected Legislative or other measures that encourage consumers to make healthier choices about industrially produced TFA	OR i. mandatory declaration of TFA on nutrition labels, OR ii. front-of-pack labelling system that includes TFA, OR iii. Reformulation or mandatory limits on industrially produced TFA in foods in specific settings (e.g., public institutions) or specific products (e.g., complementary foods).	
Comprehensive implementation / enforcement of trans fatty acid policies	i. Monitoring system for transfatty acids implemented • Laboratory testing or label assessment of food items sampled at food outlets; (could include label assessment tool) OR • Inspection of manufacturing facilities and at ports of entry. ii. Effective enforcement system implemented : • systematic tracking of inspections and offences (ideally in a database); penalties on violators (as appropriate for local legal system)	WHO Validation Programme for Trans Fat Elimination or relevant documents such as Monitoring/ enforcement protocols, monitoring enforcement reports,

9 WHO global report on trans-fat elimination obtains data from NCD CCS, therefore NCD CCS is the source of validation

Monitoring the progress of the implementation of SEAHEARTS interventions at primary health care facilities

Progress of the implementation of SEAHEARTS interventions at primary health care facilities

Process indicators	Hypertension (HTN) Yes/No/ Number	Diabetes (DM) Yes/No/ Number
1. Target setting and announcement		
Proposed country commitment for SEAHEARTS 2030 is accepted and target is announced		
Policy makers / programme manages are advocated for required programmatic and budgetary allocations		
Subnational level targets are set up – Provinces/ districts/ municipalities/ townships		
2. Measures to improve early detection of HTN and DM and register them for care		
The number of primary health care facilities where opportunistic screening for HTN and/or DM among adults is practiced		
If yes, The opportunistic screening for HTN and/or DM at primary health care facilities includes defined path for screened positives to access diagnosis services and enrolment for treatment services		
Screening for HTN and/or DM are being done in Government sector workplaces		
Contextualised models of screening is in operation in urban primary health care facilities		
HTN and/or DM screening and treatment is integrated with HIV/AIDS programmes		
HTN and/or DM screening and treatment is integrated with TB programmes		
Methods are in place for follow up of high-risk post-partum women for screening and treatment of HTN and/or DM		

Process indicators	Hypertension (HTN) Yes/No/ Number	Diabetes (DM) Yes/No/ Number
3. Standard treatment protocols for HTN and DM		
Nationally approved management protocols for managing HTN and/or DM at primary health care facilities available It includes • algorithms which are linear and follow one step at a time approach • drug and dose details • guidance on dose intensification /titration by the primary health care facilities staff to reduce treatment inertia • referral and back referral criteria for regular screening of complications		
The number of primary health care facilities in the country where protocol is followed		
The number of secondary/tertiary care facilities in the country where protocol is followed		
Referral and back referral as per the protocol are being practiced		
Pharmacological treatment is initiated at least within four weeks (preferably earlier) of confirmation of diagnosis of HTN		
Pharmacological treatment is initiated at least within four weeks of confirmation of diagnosis of DM		
National protocol for HTN and/or DM management at primary health care facilities include provision to use statins and aspirin according to risk level and history of CVD		
If yes, the number of primary health care facilities where provision to use statins and aspirin according to risk level and history of CVD with statins dispensed, is practiced		
National protocol for DM management at primary health care facilities include prescription of insulin		
If yes, the number of primary health care facilities where provision to prescribe and dispense insulin, is practiced		
Protocol for HTN include single combination pills		

Process indicators	Hypertension (HTN) Yes/No/ Number	Diabetes (DM) Yes/No/ Number
Protocol is in operation in the private sector in the country (as applicable in the country context)		
4. Access to essential medicines and devices		
HTN and/or DM medicine forecasting is based on a morbidity/ mixed based system		
All medicines of protocols to be used in primary health care facilities are included in the National Essential Medicines Lists (NEML) of the country		
Statins are included in the NEML		
Insulins are included in the NEML		
Medicines are delivered in blister packs in primary health care facilities		
Single combination pills are registered to be used in the country		
A streamlined system with the support of procurement agency, regulators and supply chain management to make medicine availability consistent		
Protocol medicines procured volume per year is sufficient covering the target population		
A system in place for monitoring of stockouts at primary health care facilities levels and for corrective actions		
The number of primary health care facilities recording stockouts of protocol medicines for HTN and/or DM in the past three months		
BP measurement devices at primary health care facilities are automated, professional and clinically validated		
If yes, the number of primary health care facilities where automated, professional and clinically validated BP devices are used		
Plasma glucose measurement is available at primary health care facilities through laboratory-based method or point-of care testing		
If yes, the number of primary health care facilities offering plasma glucose measurement (venous blood) is available at primary health care facilities through laboratory-based method or point-of care testing		

Process indicators	Hypertension (HTN) Yes/No/ Number	Diabetes (DM) Yes/No/ Number
HbA1C testing facilities are available at primary health care facilities / provisions are available for HbA1C testing at primary health care facilities through laboratory support system		
If yes, the number of primary health care facilities where HbA1C testing facilities are available at primary health care facilities / provisions are available for HbA1C testing at primary health care facilities through laboratory support system		
5. Team based care and Task sharing		
Staff (non-physician providers) at primary health care facilities are engaged in the tasks to reduce the workload of doctors		
Health workers trained for accurate blood pressure and blood glucose measurement with provision of regular refresher training at primary health care facilities		
A system of supportive supervision in place to build the capacity of primary health care facilities teams		
A system of clinical audits in place to ensure quality of services		
6. Promote continuity of services through patient centred services		
Protocol medicines are offered free/through the health benefit packages with no cost to the patient at the point of receipt		
Essential tests for DM diagnosis are offered free/ through the health benefit packages with no cost to the patient at the point of receipt		
Multi- month prescriptions (longer refill) for controlled patients		
Training videos/material in local language on BP and blood glucose self-monitoring skills to be made available on websites and social media handles		
A system to track the patients who has defaulted clinics (missed visits) as per National management protocol		

Process indicators	Hypertension (HTN) Yes/No/ Number	Diabetes (DM) Yes/No/ Number
Laboratory tests required by the protocol for monitoring of the controlled status/ complications are offered at the primary health care facilities on the same day • Fasting plasma glucose/HbA1C • Total cholesterol • Urine ketones • Serum creatinine		
A system is in place to counsel patients regularly for medication adherence		
Existence of patient support groups /peer groups for shared learning		
7. Information systems to monitor patient and programmatic outcomes		
Provision to longitudinally track people using unique ID based to avoid multiple counts on protocol-based management using digitalised systems at primary health care facilities level		
If yes, the number of primary health care facilities in which the above system is in place		
Number of primary health care facilities using electronic based record system for patient tracking and monitoring		
Digitalised NCD information systems are inter-operable with secondary and tertiary care levels or system to avoid registering same person for same condition at different facilities		
Digital dashboard are used at primary health care facilities to visualise facility-based data and are being used for monitoring of own performance on HTN and DM coverage and control		
The number of primary health care facilities with digital dashboard to visualise facility-based data and are being used for monitoring of own performance on HTN and DM coverage and control		
Digital dashboard is used at district level to track performance on HTN and/or DM coverage and control		

Process indicators	Hypertension (HTN) Yes/No/ Number	Diabetes (DM) Yes/No/ Number
Digital dashboard are used at national level to track own performance on HTN and/or DM coverage and control or integrated in Health management information systems		
Private sector share data on HTN and/or DM treatment coverage and control		
8. Measures to promote accountability		
Core HTN and/or DM indicators used for regular performance review at facility, subnational and national levels		
Systems for appraisal of performance to recognise high/low performing facilities, districts (checklist with scores or any other method)		
Regular publication of progress reports to drive accountability and to foster a continuous learning		

World Health Organization

South-East Asia Region

9 789290 221869